WRITING SAMPLE · POLICY ANALYSIS & PROGRAM EVALUATION

Ten-Year Chronology of Screening Policy Change

A version-by-version review of the policies governing intake, supervisory review, statutory definition and screen-out decisions, 2014–2024.

DOCUMENT	**Policy chronology**
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ORIGINAL DATE	**July 2024**

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The following is a chronological explanation of policy changes between 2014 and 2024 affecting the screening of child abuse and neglect reports. Information was gathered from a review of archived policies and current policy postings.

2.15 — Hospitalized victim of child abuse/neglect

Effective 1 September 2008, the Indiana Department of Child Services (DCS) authorizes the release of a child from the hospital under specific circumstances. First, if the child is the subject of a child abuse and/or neglect (CA/N) assessment. Second, if the child is currently a patient in the hospital. Third, if the hospital either reported the CA/N assessment or has been notified about it. In such cases, DCS provides the hospital with a written release or a court order, allowing the child to be released to their parent, guardian, custodian, resource parent, or court-approved placement. If DCS initially provides a verbal release, it follows up with a written release or court order on the next business day to confirm authorization for the child's release.

This policy was revised 1 October 2022 to the newest policy format.

2.27 — Community Child Protection Team

Versions 1 and 2 fall outside the ten-year timeframe and are not analyzed here.

This policy establishes the Community Child Protection Team (CPT), which serves as an oversight body composed of community members. Its primary responsibilities include reviewing cases in which DCS has been involved within the county where the CPT operates. The team also addresses complaints related to CA/N cases brought forward by individuals or agencies. Importantly, if the CPT believes it is in the best interest of the child, it may recommend that DCS file a petition in juvenile court on behalf of the subject child.

- **Version 3** was effective 1 January 2013 to 31 December 2019.
- **Version 4** was effective 1 July 2020 to 31 March 2021. The key change regarding screen-in / screen-out between these two versions is that the function of reviewing CA/N reports that have been screened out is highlighted more clearly as an optional activity.
- **Version 5** was effective 1 April 2021 to 30 September 2021. Major changes revolve around clarity of the policy. The function of reviewing screened-out reports remains explicitly stated as optional, as in the prior version.
- **Version 6** was effective 1 October 2021 to 31 October 2023. Major changes were imposed as a result of a Rapid Improvement Event. In this version the optional function of reviewing CA/N reports that have been screened out is omitted entirely. The focus of the CPT is streamlined to two main functions: reviewing cases DCS has been involved

in, and reviewing complaints regarding CA/N cases. There is also a change regarding complaint review, noting that complaints may be brought by a person or an agency.

- **Version 7** is effective as of 1 November 2023 and was revised to the newest policy format.

3.05 — Supervisory review of child abuse/neglect reports

Versions 1 to 4 fall outside the ten-year scope.

This policy mandates that each CA/N intake report undergoes review and approval by a DCS Hotline Intake Supervisor, ensuring careful assessment of the report's content. The supervisor evaluates the available facts to assess the safety of the alleged child victim. Based on this review, recommendations are made, including assigning the report for further investigation, referring it to another state if necessary, or screening it out if the allegations do not meet the statutory definition of child abuse or neglect. Specific exceptions apply, and reports involving suspected head or neck injuries receive additional attention.

- **Versions 5 and 6**, effective 1 January 2014 to 31 December 2014 and 1 January 2015 to 29 February 2016 respectively, are not changed by legislation but add clarity on specifics for pediatric referrals for children under 10 years of age with injury or suspected injury to the head or neck, and all children under three years of age with fractures or burns or suspected fractures or burns.
- **Version 7**, effective 1 March 2016 to 30 June 2017, considers legislative changes in its revisions. It lowers the threshold for mandatory pediatric referrals to children under 6 years of age with injury or suspected injury to the head or neck. Other policies and procedures remain largely the same, ensuring consistency in the supervisory review and handling of CA/N intake reports.
- **Version 8**, effective 1 July 2017 to 30 June 2019, also considers legislative changes. The primary change is the introduction of a discretionary bypass for Hotline Supervisor review, allowing DCS management to decide whether a report needs to undergo supervisory review. Other elements remain consistent with the previous version.
- **Version 9**, effective 1 July 2019 to 31 March 2023, introduces new changes not linked to legislation. Key changes include timing adjustments, which moved the time frame for certain CA/N assessments from 1 hour or 24 hours to 2 or 24 hours. Terminology for the system used to flag pediatric referrals was also simplified to “case management system”.
- **Version 10**, effective 1 April 2023 to 31 October 2023, considers changes from a Rapid Improvement Event. These changes detailed the review process for Intake Specialist recommendations and the possibility of overturning decisions, explicitly stated that the final recommendation will be made at the local office level, and added a specific

procedure for notifying relevant individuals about CA/N fatality and near-fatality intake reports.

- **Version 11** has been effective since 1 November 2023. Key changes include the addition of specific requirements for notifying parents or guardians and clarification of the timing, reiteration that the final recommendation is made at the local office level, and clarification of the timing for notifications regarding CA/N fatality and near-fatality reports.

3.06 — Recommending child abuse/neglect reports for screen-out

Versions 1 to 3 fall outside the ten-year timeframe and are not listed here.

This policy requires that when an intake report is "screened out," DCS does not recommend it for assignment as an assessment. This determination is made based on specific criteria. The DCS Child Abuse and Neglect Hotline is responsible for making the initial recommendation.

- **Version 4**, effective 1 January 2014 to 20 September 2021.
- **Version 5**, effective 1 October 2021 to 31 May 2024.
- **Version 6**, effective 1 June 2024.

3.08 — Statutory definition of child abuse/neglect

This policy outlines the process for determining whether allegations meet the statutory definition of CA/N. It involves a careful, balanced assessment of both objective and subjective information. The paramount consideration remains the safety, permanency, stability and well-being of the alleged child victim.

- **Version 3** was effective 1 January 2014 to 30 June 2016.
- **Version 4**, effective 1 July 2016 to 30 June 2017, considers legislative changes and broadens the criteria to include trafficking offenses, providing a more detailed and specific framework for identifying cases that warrant further investigation.
- **Version 5**, effective 1 July 2017 to 30 June 2018, considers legislative changes. The revision introduces new CHINS categories covering a wider range of issues, including missing children, disabilities and prenatal substance exposure. It also revises existing CHINS definitions to ensure they are comprehensive and cover various scenarios that may indicate CA/N. The addition of new CHINS categories 6 through 11 broadens the criteria for screening in cases. Revised CHINS 10 and 11 now specifically address prenatal substance exposure and related developmental issues, which were not explicitly covered before. For screen-out procedures, cases that do not meet any of the new or revised CHINS definitions may be more likely to be screened out. The revised

criteria provide clearer guidance for determining which cases meet the statutory definition of CA/N, potentially leading to more consistent decisions.

- **Version 6**, effective 1 July 2018 to 30 June 2019, considers legislative changes. The revision introduces more specific and detailed CHINS definitions, particularly related to severe offenses and conditions, and refines the criteria for determining whether a case should be screened in for further assessment, providing clearer guidance for Intake Specialists and Supervisors. The inclusion of specific criminal offenses and the broader criteria for CHINS 2 and 3 increase the likelihood of cases being screened in for assessment. The new definitions make it clearer when a case should be assigned for further evaluation based on severe offenses or conditions. In terms of screen-out procedures, this revision introduces a narrower scope, in that cases which do not meet the newly expanded or revised CHINS definitions may be screened out more effectively. The updated criteria help ensure that only cases fitting the statutory definitions proceed to assessment.
- **Version 7**, effective 1 July 2019 to 31 January 2021, maintains most of the previous definitions but adds clarity and detail to some criteria, particularly in CHINS 1 and 2. The updates provide clearer guidance for determining which cases should be screened in for assessment, improving the accuracy of the process. The detailed conditions in CHINS 1 and specific offenses in CHINS 2 help identify more precise cases of serious concern, increasing the likelihood of cases being screened in for further evaluation.
- **Version 8**, effective 1 February 2021 to 30 June 2021, considers legislative changes and retains most of the criteria from previous versions. This version emphasizes integration with the Structured Decision Making tool, which supports more accurate determination of cases that meet the statutory definition for CA/N and helps inform decisions on which cases to screen in for further evaluation.
- **Version 9**, effective 1 July 2021 to 17 March 2022, considers legislative changes and emphasizes a balanced and thorough assessment of allegations, incorporating a detailed procedural framework for evaluating and deciding CA/N cases. It includes a provision for the Hotline Intake Supervisor to have authority to overturn decisions, ensuring additional oversight and consistency in the screening process.
- **Version 10**, effective 18 March 2022, considers changes introduced by a LEAN event. This version maintains the procedural and evaluative framework established in Version 9 but provides additional definitions and clarifications to support more accurate and consistent application of the policy. The core procedures for screen-in / screen-out decisions remain unchanged, focusing on ensuring the safety and well-being of the child while expanding the understanding of relevant terms and roles.
- **Version 11**, effective 1 July 2024 to 2 July 2024, considers legislative changes offering a more structured and detailed approach to determining whether allegations should be screened in or out, with clearer guidelines and roles for handling these decisions. There is a clearer distinction of roles and responsibilities between the Intake Specialist

and the Hotline Intake Supervisor, with specific guidelines on when and how decisions can be overturned. There is also added emphasis on handling cases involving new children in households with open DCS cases, which affects the final recommendation for assessment or screening out.

- **Version 12** has been effective since 3 July 2024 through an administrative letter change. Version 12 again includes more detailed criteria and definitions for various forms of abuse and neglect, focusing especially on specific offenses and conditions. This revision also specifically directs that new child cases in homes with open DCS cases must be assigned for assessment, not screened out.

3.A Tool — Hotline intake overview chart

Version 1, effective 1 November 2005 to 31 August 2019, is a flowchart depicting the process for intake. It depicts three pathways:

1. **CA/N allegations** — either screened out, assigned for investigation, or transferred.
2. **Service requests** — split into self-referrals or third-party referrals.
    - Self-referrals may be assigned to a Family Case Manager for service referral, or the hotline worker may give the caller contact information.
    - In addition to self-referral possibilities, hotline workers may also ask the third party to ask the family to contact DCS.
3. **Other** — information only; adoptive and foster inquiries; licensing; collateral information; complaints; wrong numbers. All are routed according to need.

Version 2, effective 1 September 2019 to 1 March 2023, updates the format and provides additional routing possibilities. It designates the same first three routing options but updates the process after need:

1. **Service request** — split into four possible categories: self-referrals, third-party referrals, safe haven, and courtesy interview.
    - Self-referrals are assigned to a Family Case Manager for service referral.
    - Third-party referrals are assigned to a Family Case Manager for service referral.
    - Safe haven is routed to the DCS local office.
    - Courtesy interviews are routed to the DCS local office.
2. **CA/N allegations** — split into assessment recommendations and screen-out recommendations. Both recommendations may be further agreed or disagreed with by the county.
3. **Other** — examples are provided and then directed to family evaluation or information and referral. Both options are routed according to need.

Version 3, effective 1 April 2023, makes minor changes to the formatting and splits the service referral category into two categories, giving the flowchart four total intake possibilities:

1. **CA/N allegation** — same process designated in Version 2.
2. **Service request** — directed to safe haven.
3. **Professional service request** — requests from law enforcement, prosecutor and court; courtesy interviews; or out-of-state child service agency requests.
4. **Other** — same process designated in Version 2.

4.30 — Institutional Child Protection Services unit assessments

Version 1 falls outside the ten-year scope of this analysis.

- **Version 2**, effective 1 February 2008 to 31 January 2014, indicated that the DCS local office will conduct an assessment of a report of possible CA/N that occurred in an institution located within the county. Institutions include: (1) resource family homes, (2) licensed childcare homes and centers, (3) public and private schools, (4) hospitals, (5) group homes, (6) residential treatment centers, (7) emergency shelter care centers, (8) correctional facilities, and (9) unlicensed registered child care ministries (new in this version).
- **Version 3**, effective 1 February 2014 to 30 July 2014, added that the Institutional Child Protection Services (ICPS) unit would conduct assessments of reports of CA/N if allegations state the incident occurred in the care of one of the following: (1) residential facility, (2) school, (3) hospital, (4) juvenile correctional facility, (5) adult correctional facility that houses juvenile offenders, (6) Bureau of Developmental Disabilities certified group home, (7) licensed child care home or center, (8) unlicensed registered child care ministry, or (9) unlicensed child care home or center. It also stated that DCS would not conduct an assessment on an unlicensed registered child care ministry without Law Enforcement Agency involvement unless the ministry received Child Care Development Fund vouchers. Additionally, the ICPS unit would not assess CA/N reports on foster homes or fatalities and near fatalities — local DCS offices would assess these. DCS and ICPS coordinate on any assessment involving a fatality or near fatality that occurs in an institutional setting.
- **Version 4**, effective 1 July 2014 to 31 July 2018, revised procedures to add that the ICPS Family Case Manager would provide a copy, upon request, of the approved Assessment of Alleged Child Abuse or Neglect Report to the director of a residential facility when a CA/N assessment is completed, if the allegations involved a facility employee or volunteer when the child was placed at the facility.
- **Version 5**, effective 1 August 2018 to 30 June 2019, added procedures related to supervisory approval of the safety plan and regular supervisory staffing with the ICPS

Family Case Manager.

- **Version 6**, effective 1 July 2019 to 31 July 2021, added minor changes and adjustments in wording, including a shift to “the case management system” and changes in the titles of referenced policies.
- **Version 7**, effective 1 August 2021 to 30 June 2024, included minor changes such as the addition of policy references and changes of abbreviations to reference forms 310 and 311.
- **Version 8**, effective 1 July 2024, was revised in accordance with legislative changes. This version makes changes to policy formatting and to procedure. Notably, it states that DCS ICPS will request joint assessments with a Law Enforcement Agency for licensed child care homes, residential childcare centers, or schools, or employees or volunteers of those persons or entities. Additionally, the ICPS unit will conduct assessments on reports of CA/N if the alleged victim is at least 18 but less than 21 years of age and resides or has previously resided at a DCS-licensed residential facility and is harmed or threatened with harm by a member of the facility staff as a result of a battery offense or sexual activity. Specific to screen-outs, this policy also adds that ICPS supervisors will send a copy of all form 310s recommended for screen-out to the Law Enforcement Agency.

17.13 — Review for licensing action

Effective 1 July 2022, this policy states that the DCS Residential Licensing Unit conducts a review of concerns for the safety of children receiving treatment from a licensed residential facility, and of any facility noncompliance with Indiana Code, Indiana Administrative Code, DCS policies, or the facility's contract. The review may be in addition to regularly scheduled annual licensure reviews and may result in the requirement of a Plan of Correction.

The Residential Licensing Unit may receive information regarding concerns for a licensed and/or contracted residential facility from sources including, but not limited to:

1. Annual or relicensure review
2. Background check review
3. Institutional Child Protective Services unit investigations
4. Preliminary Report of Alleged Institutional Child Abuse or Neglect (form 310A) which has been screened out
5. Complaint from another agency
6. Incident reports, including critical incidents
7. Root cause analysis

Administrative Letter DCS-21-08 — Screen-out reports

This administrative letter, effective 21 May 2021 to 13 October 2021, resulted from a Rapid Improvement Event in April 2021. The letter changed the screen-out report process such that, as of 1 June 2021, every screen-out report was to be reviewed by one hotline supervisor and then sent to local DCS offices for one member of management to review the report and make a final determination within 24 hours.